



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نادية محمد على الصيرفي
المحافظة : القاهرة
الحى / المدينة : حلوان
رقم البطاقة : 26810210102248 رقم التليفون : 01012210212
العمر (تاريخ الميلاد) : 21-10-1968 المهنة : ربه منزل
العنوان : 15 مايو مجاوره 22
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]
قسم العلاج :
الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نادية محمد علي الصيرفي
National ID: 26810210102248



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين


 دولة الكويت
 وزارة الداخلية
 عائلة علي النصفية
 تاريخ الميلاد: ٢٠٢٢/٠١/٢١
 رقم الهوية: ٢٦٨١٠٢١٠١٠٢٢٤٨
 رقم البطاقة: JF0903363


 دولة الكويت
 وزارة الداخلية
 عائلة علي النصفية
 تاريخ الميلاد: ٢٠٢٢/٠١/٢١
 رقم الهوية: ٢٦٨١٠٢١٠١٠٢٢٤٨
 رقم البطاقة: JF0903363

MDT FORM

Patient Name			
Sex	F	Age	57
Family History	-	Special habits	-
Surgical History	-	Comorbidity	-
Menopausal Status	postmenopausal	Telephone	010122101212
ID	26810210102248	Marital Status	
Date	12-11-2025	Medical ID	3099/2025
		Reason of MDT	

Medical Report

Pathology: IDC grade II

ER: 4/8 PR 6/8 HER2: -VE Ki67: 10%

pT2 N3 M

N: 10/12

S.M: upfront surgery EOS & ALND (21/10/2025)

Metsworkup:

Doctors' notes:

Recommendations:

For slide review → requested

Pet scan → ???

To start AC x4 → taxol x4 → PORT → AI x7 → zometa for 6 months

MDT DECISION

T2N3M0 L.A

For ACX3, TXT X3, PORT, AI

Name

Signature

أحمد عبد الله

12/11/25

منى زكي

12/11/25

M. Kelay
12/11/25

Moner
12/11/25

12/11/2025

Name: ناديه محمد علي		
MR. : 5696		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a 57-year-old individual with a history of diabetes mellitus, hypertension, and prior mastectomy. The patient was diagnosed with left breast cancer, staged as PT2 N3 (10/21), estrogen receptor 4/8 positive, progesterone receptor 6/8 positive, and HER2-negative on PET-CT. The treatment plan was outlined as three cycles of AC (doxorubicin and cyclophosphamide) followed by three cycles of TXT (taxane). The patient completed the adjunctive TXT today in conjunction with femara and was supported with a three-month PORT; the PORT is pending. The PORT was discontinued in June 2026; femara was administered today, and a three-month supply of medication was arranged through the pharmacy.		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	
علياء احمد أبو النصر شهاب الدين		الطبيب المعالج :
يعتمد ,		

Patient Name	: الأستاذ / نادية محمد علي الصيرفي	Request No.	: 13009445
Patient No.	: 26810210102248	Request Date	: 27/10/2025
Path No.	: SR-1518/25	Verification Date	: 10/11/2025
Sex	: Female	Financial Category	: نفقة الدولة - جوستاف روسي
Age	: 57 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data

Left breast mass.

Nature of specimen

Block and slide revision

Gross examination

Ten referred paraffin blocks labelled by (8810/25) and seventeen slides were received.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

Excision :

Histological type: Invasive duct carcinoma.

* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2

* DCIS: Positive , cribriform &solid subtypes . 5%

* Lobular intra epithelial neoplasia : Absent

* Angioinvasion: Positive

* Microcalcification :Positive

* Perineural invasion: Absent

* Lymph nodes :Positive in ten out of twenty one lymph nodes (10/21)

* Margins : Free

CORE :

* Histological type: Invasive duct carcinoma with micropapillary features

* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2

* DCIS: Positive , cribriform &papillary

* Lobular intra epithelial neoplasia : Absent

Verified by :

Dr.Heba Hasan

Printing Date : 06/01/2026 13:14:32

Page 1 of 3

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / نادية محمد علي الصيرفي	Request No. : 13009445
Patient No. : 26810210102248	Request Date : 27/10/2025
Path No. : SR-1518/25	Verification Date : 10/11/2025
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 57 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- * Angiolinvasion: Absent
- * Microcalcification :Positive
- * Perineural invasion: Absent .
- * IPI / Positive control showed appropriate reactivity) :
- * ER:Weak nuclear reaction in 30 % tumor cells (Allred score 4/8)
- * PR: Moderate nuclear reaction in 40 % tumor cells (Allred score 6/8)
- * HER2: No membrane reaction in tumor cells (0)
- * Ki67: 10 %

Diagnosis

LEFT BREAST MASS, block and slide revision :

Excision :

- Invasive duct carcinoma Grade 2 (tubular differentiation 3, pleomorphism 2, mitosis 1) (according to Nottingham grading system of breast cancer) ,
- DCIS , cribriform & solid subtypes 5 %.
- Free margins
- Axillary lymph nodes :positive in ten lymph nodes out of twenty one lymph nodes (10/21) with extracapsular extension .

Core :

- Invasive duct carcinoma Grade 2 (tubular differentiation 3, pleomorphism 2, mitosis 1) (according to Nottingham grading system of breast cancer)with micropapillary features ,
- positive DCIS , cribriform and papillary subtypes .

IPI:

- * ER: Positive (score 4/8).
- * PR: Negative (score 6/8).
- * HER2: Negative (score 0)
- * Ki67: 10 %

Verified by :

Dr.Heba Hasan

Printing Date : 17/11/2025 13:49:52

Page 2 of 3

Printing By : Mrs.Mayada Hegazy

Patient Name : الأستاذ / نادية محمد علي الصيرفي

Patient No. : 26810210102248

Path No. : SR-1518/25

Sex : Female

Age : 57 Y

Referreing Doc :

Request No. : 13009445

Request Date : 27/10/2025

Verification Date : 10/11/2025

Financial Category : نفقة الدولة - جوستاف روسي

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Printing Date : 06/01/2026 13:14:32

Page 3 of 3

Printing By : Ali mostafa



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Dd' Etat C E S Path (Paris), D.A.E. (Paris)
Fellow Johns Hopkins Medical School (USA)

Dr. Mohab S. Eissa, M. D.

M.D. Pathology Oncology
Pathology Consultant

دكتور سعد سيد عيسى

دكتورة باثولوجى - باريس
زميل جامعة جونز هوبكنز - الأمريكية

دكتور مهاب سعد عيسى

دكتورة باثولوجيا الاورام
إستشارى باثولوجيا

PATHOLOGY REPORT

Name: NADIA MOHAMED ALI

Age: 57 years

Clinical picture: Left Breast Cancer

Nature of specimen: ER, PR and HER-2 and KI-67 Immunohistochemistry

Referred by: Prof. Dr. MOHAMED GAMAL AWAD

Date: 18 / 9 / 2025

Gross:

Estrogen and progesterone receptors, Her-2/neu and KI-67 performed on unstained sections of the tumor paraffin using monoclonal antibodies (Dako cytomation USA) and immunoperoxidase method. A positive stain appears as dark brownish granules in the nuclei of the tumor cells (ER and PR) and as membranous stain (Her-2/neu) as follows:

Score to report	HER2 protein overexpression assessment	Staining pattern
0	Negative	No staining is observed, or membrane staining is less than 10% of tumor cells
1+	Negative	A faint/barely perceptible membrane staining is detected in more than 10% of the tumor cells. The cells are only stained in part of the membrane
2+	Positive	A weak to moderate complete membrane staining is observed in more than 10% of the tumor cells.
3+	Positive	A strong complete membrane staining is observed in more than 10% of the tumor cells.

Microscopic:

The tumor cells stained moderate positive for estrogen receptors (ER in 35% of the nuclei of tumor cells with 6/8 score and strong positive for progesterone receptors (PR) in 50% of nuclei of tumor cells with score 7/8. The tumor cells stained negative for HER-2 over expression (score 0). The KI-67 labeling index 8%.

Diagnosis:

- 1- ESTROGEN RECEPTORS STATUS:
- 2- PROGESTERONE RECEPTORS STATUS:
- 3- HER-2/NEU IMMUNOHISTOCHEMISTRY:
- 4- KI-67 LABELLING INDEX:

POSITIVE (+2)
POSITIVE (+3)
NEGATIVE (score 0)
(8%)

(Code: 416 / 9 / 2025)

Date of issue: 25 / 9 / 2025

معمل : ٢٨١٤٠١٨ - ٢٤٥٠٥٧٢٤
مكتب : ٢٨١٠٨١٠٥

معمل ١٢٠٦ بطرمس على متفرع من إبراهيم اللقاني ميدان روكسى - مصر الجديدة - النور الخامس

E-Mail : drsaadeissa@hotmail.com

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دكتور مهاب سعد عيسى

دكتورة باثولوجيا الاورام
إستشارى باثولوجيا

PATHOLOGY REPORT

Name: NADIA MOHAMED ALI

Age: 57 years

Clinical picture: Left Breast Mass

Nature of specimen: 1- Left breast US Guided Core Biopsy and Cell Block, 2- US Guided FNAC

Referred by: Prof. Dr. MOHAMED GAMAL AWAD

Date: 18 / 9 / 2025

Gross:

- 1- Four cores from left breast processed and stained with H&E.
- 2- Cell block stained with H&E
- 3- Six smears stained with papanicolaou stain

N.B. No core of the axillary lymph node biopsy obtained.

Microscopic:

- 1- The core biopsy revealed invasive duct carcinoma grade 2. The tumor is formed of solid groups of malignant duct cells with enlarged irregular hyperchromatic nuclei showing moderate anaplasia and infrequent mitosis. The tumor infiltrated the mammary stroma. No ductal carcinoma insitu component (DCIS) noted.
- 2- The slides of the cell block revealed blood clots, material insufficient for diagnosis.
Five smears of breast FNA revealed exfoliated clusters of malignant duct cells with enlarged hyperchromatic nuclei, increased N/C ratio and nuclear molding
- 3- The six smears revealed lymph node showing metastatic deposits of adenocarcinoma cells.

Diagnosis:

LEFT BREAST US GUIDED CORE BIOPSY AND FNA AND AXILLARY LYMPH NODE
FNAC: POSITIVE FOR INVASIVE DUCT CARCINOMA GRADE 2 WITH AXILLARY LYMPH
NODE METASTASIS.

THE ER, PR, HER2 AND KI67 WILL BE DONE ON BREAST CORE BIOPSY AND RESULT
WILL BE GIVEN ON SEPARATE REPORT.

(Code: 416 / 9 / 2025)

Date of issue: 24 / 9 / 2025

Dr. Saad S. Eissa, M.D.

Dd' Etat C E S Path (Paris), D.A.E. (Paris)
F.L.E.X. (Maryland, USA)

Former Fellow Johns Hopkins Medical School
Member of Societe Anatomique de Paris
Member of International Academy of Pathology
Professor and chairman, Pathology department
National Cancer Institute, Cairo University

دكتور سعد سعيد عيسى

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رئيس مكتبة طب جونز هوبكنز الأمريكية
عضو الأكاديمية الدولية الباثولوجية
مستشار ورئيس قسم التحاليل الباثولوجية والأورام
معهد الأورام - جامعة القاهرة

PATHOLOGY REPORT

Name: NADIA MOHAMED ALI EL SERAFY

Age: 56 years

Clinical picture: Left Breast Cancer

Nature of specimen: Left Breast Conservative Breast Surgery and Axillary Clearance with Frozen Section Control

Referred by: Prof. Dr. MOHAMED GAMAL

Date: 5 / 10 / 2025

Gross:

1,2- Left Breast UOQ mass, Invasive duct carcinoma (Luminal) subjected to Frozen Section Control: Confirmed invasive duct carcinoma with free resection margins, wide local excision with extra circumferential margin & axillary clearance.

1,2- Fibro-fatty tissue segment 12x10x8 cm with covering skin 8x2.5 cm & anatomically orienting sutures. Cut-sections reveal ill-defined firm mass 3x2.5x2.3 cm. Superior 3 cm, Inferior 2.7 cm, Medial 4 cm, Lateral 3.5 cm & Deep 2.5 cm.

3- Multiple suspicious lesions, Fibro-fatty pieces 2x2 cm, totally submitted

4- Reduction Mammoplasty, Fibro-adipose tissue piece 4x3.5 cm, totally submitted

5- Axillary clearance, Axillary tail 7x7 cm entangling multiple lymph nodes ranging from 0.5x0.5 cm to 2.2x2 cm.

Microscopic:

Sections examined revealed multifocal invasive mammary carcinoma and ductal carcinoma insitu. Invasive carcinoma is formed of irregular solid groups of malignant epithelial cells with ductal differentiation surrounded by desmoplastic stroma. Tumor cells show moderate anaplasia and focally increased mitosis. Tumor lympho-vascular emboli were detected. Sections from the suspicious lesion showed insitu component of solid and cribriform types, intermediate grade. Margins of excision are free of tumor cells.

Axillary lymph nodes are positive for tumor macrometastasis (10/21), with extracapsular invasion.

Diagnosis:

LEFT BREAST, BAT WING REDUCTION MAMMOPLASTY AND AXILLARY LYMPH NODES CLEARANCE WITH FROZEN SECTION CONTROL: MULTIFOCAL INVASIVE DUCT CARCINOMA, GRADE 2 AND DUCTAL CARCINOMA INSITU, FREE RESECTION MARGINS. TWO POSITIVE AXILLARY LYMPH NODES (10/21). STAGE: PT2N3.

(Code: 88 / 10 / 2025)

Date of issue: 7 / 10 / 2025



Imaging report

Pat. Name: Nadla Mohamed Ali Al Serafi



كايرو سكان
للشعة والتشخيص
CAIRO SCAN
Radiology & LMS

Pat. ID: 3406753

Date: 21/09/2025

F-18 Fluorodeoxyglucose (FDG) PET/CT of the whole body:

Clinical History:

57 years old female with left breast mass for assessment .

Procedure:

- **Dose/Radiopharmaceutical:** F-18 Fluorodeoxyglucose (FDG) was administered via a peripheral upper limb vein.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- **A low-dose non-contrast CT images** for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the thighs.
- **A diagnostic post-contrast CT examination** of the same regions was performed after IV administration of non-ionic contrast.
- **PET/CT Images** were reviewed in the transaxial, coronal and sagittal planes.
- **The Standardised Uptake Value**, maximum variant (SUV max) was calculated within regions of interest whenever needed

Findings:

Head and neck:

- The brain appears normal with no abnormal focal metabolic activity detected.
- No adenopathy or nodal hypermetabolism in the neck.
- Normal FDG uptake pattern is seen at the soft tissues of the neck.
- Physiologic radiotracer uptake is demonstrated at the oropharynx, larynx and salivary glands.
- Bilateral maxillary sinusitis.

Chest:

- low grade metabolically active left breast small nodular mass 13 X 12 mm with SUVmax 1.9, likely neoplastic . for sonomammography.
- low grade metabolically active multiple enlarged left axillary lymph nodes, largest being 13 mm. in diameter with SUVmax 1.5 , likely metastatic

Imaging report



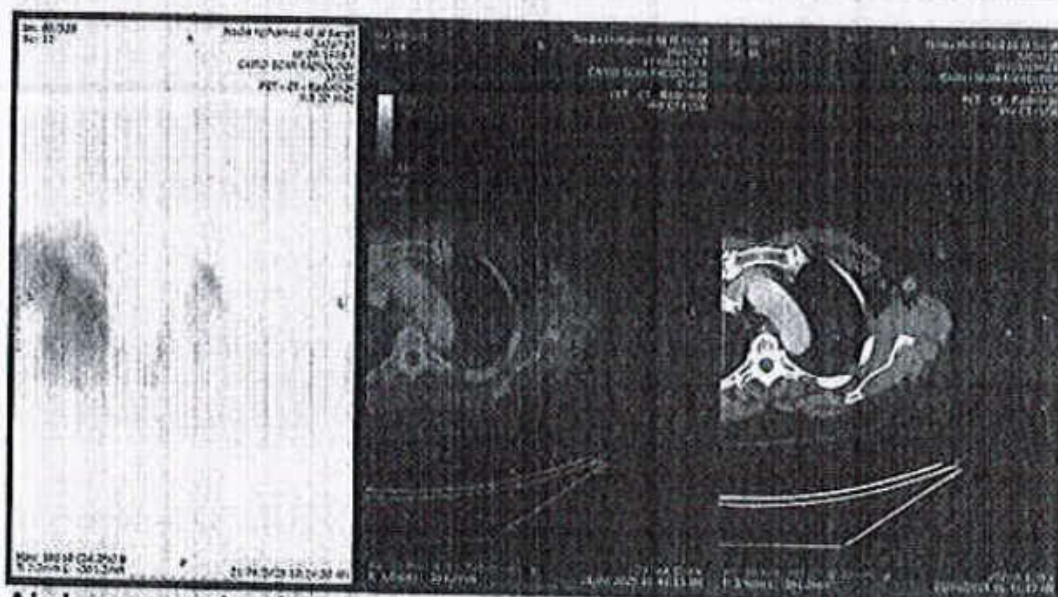
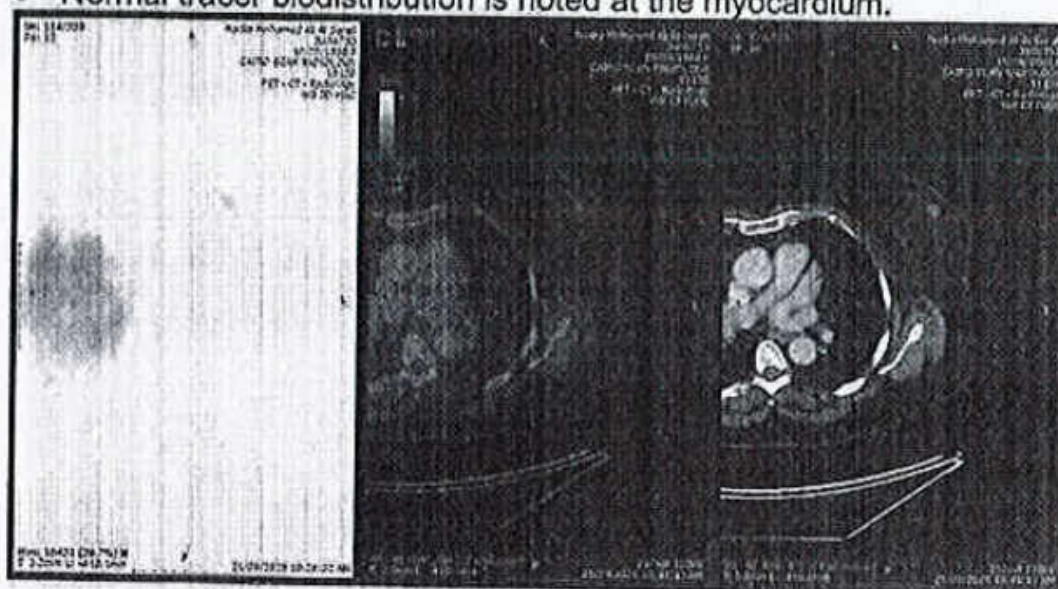
كايرو سكان
للإشعة والتحاليل
CAIRO SCAN
Radiology & Labs

Pat. Name: Nadia Mohamed Ali Al Serafi

Pat. ID: 3406753

Date: 21/09/2025

- No metabolically active pulmonary nodules or other parenchymal abnormalities demonstrated.
- No metabolically active or pathologically enlarged right axillary, supra-clavicular, hilar or mediastinal lymph nodes could be detected.
- No pleural or pericardial collections.
- Normal tracer biodistribution is noted at the myocardium.



Abdomen and pelvis:

- Liver is enlarged (20 cm. in MCL) showing homogenous texture and smooth outline.

أ.د. ناجح عبد الوهاب

أستاذ الأشعة - جامعة القاهرة

تاريخ الفحص: 18/9/2025

كود المريض: 675952

رقم العملية: 11250917012

Thank you for referring this case,

ناديه محمد على

57

نقابة المهندسين

الاسم:

السن:

جهة التحويل:

الطبيب المعالج أ.د.:

ULTRASOUND GUIDED TRUE CUT BIOPSY

- Procedure and possible complications were explained to the patient
- Usual sterilization of site of biopsy
- Local anesthesia (3cc) was infiltrated at site of sampling
- A few cores are taken by 14G Auto-cut needle from left breast malignant mass
- Sample immediately preserved in 10% formalin solution
- Procedure is well tolerated
- No immediate complication

Radiological picture:

- BIRADS 5: malignant mass

Required Data:

- *Histopathological confirmation*
- *ER, PR, HER, Ki67*

ULTRASOUND GUIDED FINE NEEDLE ASPIRATION

- Procedure and possible complications were explained to the patient
- Usual sterilization of site of biopsy
- Local anesthesia (1cc) was infiltrated at site of sampling
- Eight alcohol fixed slides and syringe containing cell block are prepared from aspirate of left pathological mostly infiltrated level I axillary LN

Radiological picture:

- Left breast malignant mass and two pathological level I axillary LNs

Required Data:

- *Cytology evaluation*

Best regards,

Dr. Amira H. Radwan M.D
Breast imaging consultant,
NCA Cairo University

18/9/2025
Page 1 of 1



Imaging report

Pat. Name: Nadia Mohamed All Al Serafi



كايرو سكين
بالتصوير والتحاليل
CAIRO SCAN
Radiology & Labs

Pat. ID: 3406753

Date: 21/09/2025

- No hepatic focal lesions or areas of abnormal enhancement or metabolic activity.
- No biliary radicles dilatations could be noted.
- Normal appearance of the spleen, pancreas, adrenals, both kidneys, aorta (apart from calcific atheromatous plaques) and IVC as well as pelvic organs
- Normal FDG activity is seen at the gastrointestinal and genitourinary systems, with no striking focal hypermetabolic lesions detected.
- No pathologically enlarged or metabolically active abdominal or pelvic lymph nodes.
- No ascites.

Musculoskeletal:

- Spinal spondylo-degenerative changes)
- L4/5 and L5/S1 bilateral facet arthropathy.
- Intermuscular lipoma 4.4 X 2.1 cm. in left upper thigh.
- FDG biodistribution is rather physiologic, with no significant blastic or lytic skeletal lesions could be seen on CT to account for metastatic disease to bone.

Impression:

(FDG) PET/CT whole-body study showing:

- low grade metabolically active left breast small nodular lesion, likely neoplastic with associated low grade metabolically active left axillary lymph nodes, likely metastatic, for correlation with tumor markers and histopathology & immunophenotyping
- No gross hypermetabolic lesions detected at the rest of the scanned body.

Best Regards

Nuclear medicine consultant
Prof. Dr. Maha Abd Elkareem, MD
A. professor of Nuclear Medicine,
kasr al alny, Cairo University

Radiology Consultant
Prof. Mohsen Khalil

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Patient's Name: Nadia Mohammed Ali Al-Sayrafi
Date: 2026-07-25
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Left breast heterogeneous parenchyma with skin thickening for sonomammography.
- Left axillary soft tissue sheet measuring 40 x 24 mm ? scar tissue.
- Few bilateral scattered pulmonary nodules largest measuring 8.5 mm at the left upper lobe.
- Left upper lobe and lingular subpleural ground glass density ? post radiation sequelae.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- Precarinal lymph node measuring 12 x 8 mm.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

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Opinion:

❖ Few bilateral scattered pulmonary nodules for follow up.

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